

# MLS — Sample Medical-Legal Report

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## CAUSATION OPINION

EXAMINEE: Jordan A. Miller

DATE OF BIRTH: 1986-04-12

ATTORNEY / REQUESTING FIRM: Roe & Associates

CASE / CLAIM NUMBER: CV-2026-00482

DATE OF INJURY / ONSET: 2026-02-03

JURISDICTION: Arkansas

DATE OF THIS REPORT: 6/10/2026

## PURPOSE OF REPORT

This report was requested to provide a causation opinion regarding the relationship between the L4-L5 disc protrusion with left L5 radiculopathy and the motor vehicle collision that occurred on February 3, 2026. Specifically, counsel requested an assessment of whether the injury is causally related to the collision and whether the collision aggravated any pre-existing condition.

## RECORDS AND HISTORY REVIEWED

The following records were reviewed:

- Emergency Department record from Centerton Regional ED dated 02/03/2026
- Initial evaluation by Dr. L. Schaeffer dated 02/12/2026
- MRI lumbar spine report dated 02/20/2026
- Physical therapy progress summary dated 02/20/2026 through 04/02/2026
- Procedure note by Dr. L. Schaeffer dated 04/05/2026
- Follow-up note by Dr. L. Schaeffer dated 05/20/2026

## RELEVANT HISTORY AND CURRENT STATUS

Jordan A. Miller, a 39-year-old male, presented to the emergency department following a rear-end motor vehicle collision on February 3, 2026. He reported immediate onset of neck and low back pain. Initial examination revealed tenderness and spasm in the cervical and lumbar regions, with no acute fractures noted on imaging. Nine days post-accident, he continued to experience axial neck pain and low back pain radiating into the left buttock and posterior thigh. An MRI performed on February 20, 2026, revealed a 5 mm left paracentral disc protrusion at L4-L5, contacting the left L5 nerve root. His treatment included physical therapy and an epidural steroid injection, with partial improvement noted in symptoms.

## EXAMINATION / CLINICAL FINDINGS

- Emergency Department: Tenderness and spasm in cervical and lumbar paraspinal muscles; neuro exam grossly intact.
- PM&R Initial Evaluation (02/12/2026): Reduced lumbar range of motion (~40%); positive straight-leg raise on the left at ~45 degrees; left extensor hallucis longus weakness (4+/5); decreased sensation in the left L5 distribution.
- MRI Findings (02/20/2026): L4-L5 left paracentral disc protrusion with left L5 nerve root impingement.

- Follow-up (05/20/2026): Improved but persistent reduced lumbar range of motion; positive straight-leg raise on the left at ~60 degrees; left EHL strength improved to 5-/5.

## DIAGNOSES

1. L4-L5 disc protrusion with left L5 radiculopathy
2. Cervical strain (resolved)

## OPINIONS

Based on the history, the records reviewed, the examination findings, the diagnostic studies, the mechanism and timing, and my training and experience, it is my opinion to a reasonable degree of medical probability that the L4-L5 disc protrusion with left L5 radiculopathy was caused by the incident of February 3, 2026. The reasons supporting this opinion include:

- The mechanism of injury involved a rear-end collision, which is known to cause lumbar injuries due to the sudden acceleration-deceleration forces.
- The timing of symptom onset was immediate following the collision, consistent with the development of a disc protrusion.
- The MRI findings correlate with the clinical presentation of left L5 radiculopathy, which developed after the collision.
- There is no documentation of significant pre-existing lumbar pathology that would account for the acute presentation of symptoms following the incident.

Regarding the aggravation of any pre-existing condition, the records do not indicate any prior lumbar conditions that would have contributed to the current diagnosis. Therefore, it is my opinion that the collision did not aggravate any pre-existing condition.

## ANSWERS TO LEGAL QUESTIONS

1. Yes, the L4-L5 disc protrusion with left L5 radiculopathy is causally related to the 02/03/2026 motor vehicle collision, to a reasonable degree of medical probability.
2. The collision did not aggravate any pre-existing condition, as no such condition is documented in the records.

## SIGNATURE BLOCK

[Physician signature]

[Name, credentials]

[NPI]

Date: [Date]

The opinions expressed herein are held to a reasonable degree of medical probability.